

A Randomized Controlled Trial of an Online Dialectical Behavior Therapy Skills Intervention for War-Related Distress

Or Segal¹, Hadas Hevron¹, Helene Sher², Idan M. Aderka¹, and Noam Weinbach¹

¹ School of Psychological Sciences, Faculty of Social Sciences, University of Haifa

² Department of Psychiatry, Soroka Medical Center, Beer Sheva, Israel

Objective: Living in war-afflicted regions involves long-term detrimental consequences for emotional well-being. Given the prevalence of ongoing conflicts worldwide, there is a pressing need to evaluate brief interventions that can be disseminated quickly and broadly to improve coping with war-related stress. This study examined the efficacy of an online dialectical behavior therapy (DBT) distress tolerance skills intervention for improving emotional well-being among women experiencing elevated distress amid war.

Method: The study was conducted during the Israel–Hamas war and recruited 149 women whose first-degree relatives or spouses were deployed. Participants were randomly assigned to one of three conditions: (a) radical acceptance—training in a DBT skill promoting acceptance of reality, (b) relaxation—training in DBT skills fostering body relaxation, and (c) active control—monitoring distressing events without implementing DBT skills. The intervention included an online workshop to learn the skills and three home training sessions over 1 week to practice the skills for coping with personal distressing events. Measures of emotional distress were assessed at baseline, postintervention, 3-week follow-up, and during the home training sessions.

Results: During the home training, practicing radical acceptance and relaxation skills led to improvements in negative and positive affect. However, only radical acceptance produced significant pre-to-post intervention reductions in stress, depression, and emotion regulation difficulties, with benefits persisting at follow-up.

Conclusions: A brief online radical acceptance intervention is a cost-effective and accessible tool for coping with acute war-related stress. It may offer rapid emotional support in high-stress situations like war.

What is the public health significance of this article?

This study suggests that an online dialectical behavior therapy-based distress tolerance skills intervention effectively reduces emotional distress in women experiencing high levels of war-related STS. Brief, accessible, and cost-effective online interventions can serve as a crucial first-line response during large-scale crises.

Keywords: radical acceptance, dialectical behavior therapy, war-related stress, emotion regulation, online intervention

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In times of war, individuals face significant emotional distress that greatly impacts their mental health and psychological well-being (Kurapov et al., 2023). World Health Organization research shows that people living in conflict-affected areas experience a high prevalence of mental disorders such as depression, anxiety, and

posttraumatic stress disorder (Charlson et al., 2019). High levels of real-time negative affect during war predict postwar psychological distress, including posttraumatic stress, and depressive symptoms (Lapid Pickman et al., 2021). Furthermore, postwar psychological symptoms remain high in war-affected populations many years after

Wolfgang Lutz served as action editor.

Noam Weinbach  <https://orcid.org/0000-0003-3449-5278>

Or Segal and Hadas Hevron contributed equally to this work.

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Or Segal played a lead role in writing—original draft and an equal role in

conceptualization, data curation, formal analysis, investigation, and methodology. Hadas Hevron played a lead role in writing—original draft and an equal role in conceptualization, data curation, formal analysis, investigation, and methodology. Helene Sher played a supporting role in conceptualization, methodology, and writing—review and editing. Idan M Aderka played a supporting role in formal analysis and writing—review and editing. Noam Weinbach played a lead role in conceptualization, supervision, and writing—review and editing and an equal role in formal analysis.

Correspondence concerning this article should be addressed to Noam Weinbach, School of Psychological Sciences, Faculty of Social Sciences, University of Haifa, Abba Khoushy Avenue 199, Haifa 3498838, Israel. Email: nweinbach@psy.haifa.ac.il

the war ends (Priebe et al., 2013). Therefore, it is important to assess brief interventions that can be disseminated quickly and broadly to improve coping among those who are struggling with war-related emotional distress. One promising approach to improving coping during war is to help individuals regulate their emotions more effectively.

Emotion regulation involves a range of skills and techniques aimed at understanding, managing, and modifying one's emotional responses to distressing situations (Gross, 1999). Individuals who tend to use maladaptive emotion regulation strategies to regulate their emotional experience during crises are more likely to develop anxiety and depression and are characterized by overall poorer emotional well-being (Amone-P'olak et al., 2007; El Khoury-Malhame et al., 2024). As such, empowering individuals in war-affected regions with tools to regulate their emotions effectively could help them navigate the challenges of conflict-induced negative emotions (Gross et al., 2013). For instance, a short intervention aimed at enhancing adaptive emotion regulation strategies among war-affected children was found to improve their posttraumatic, depressive, and psychological distress symptoms (Punamäki et al., 2014). However, there are limited data on short-term emotion-regulation-focused interventions for adults who are highly affected by war.

Dialectical behavior therapy (DBT; Linehan, 1993) is an empirically supported treatment for a range of psychological disorders that are characterized by severe emotion regulation difficulties including borderline personality disorder (Panos et al., 2014), anxiety (Malivoire, 2020), depression (Harley et al., 2008), and eating disorders (Bankoff et al., 2012). Research has demonstrated that DBT can be effectively utilized in various settings, including in nonclinical populations (Mazza et al., 2016; Moore et al., 2018).

Several studies have demonstrated that comprehensive DBT can improve emotional well-being among those who were affected by war (Goodman et al., 2016; Koons et al., 2001; Landes et al., 2020; Meyers et al., 2017). However, these studies primarily examined the full DBT treatment package and were conducted outside the context of active war. Given that comprehensive DBT involves learning a wide range of skills over an extended period, it may not be a feasible solution for managing real-time stress during ongoing conflict. Further research is needed to evaluate whether specific DBT components could provide more immediate relief in war-related crises.

The distress tolerance module in DBT is specifically focused on skills required for handling acute emotional distress during crisis (Linehan, 2015) and as such may be especially relevant for individuals who are highly affected by war-related distress. DBT's distress tolerance skills are broadly divided into change-based and acceptance-based skills. Change-based skills involve actions that promote immediate short-term relief of negative emotional experiences. A central change-based skill in DBT that fosters body relaxation is "TIP", an abbreviation of "Temperature", "Intense exercise", and "Paced breathing" or "Paired muscle relaxation". TIP involves actions that aim to downregulate physiological arousal by changing the body's temperature (e.g., physically cooling the body), engaging in short intense exercises, breathing slowly and efficiently, and practicing paired muscle relaxation (Linehan, 2015; Linehan & Wilks, 2015). The short-term relief as a result of downregulating physiological arousal aims at preventing actions that would further escalate the already distressing situation (Linehan, 2015). Given the acute stress individuals experience during war, managing

physiological arousal more effectively may prove beneficial for emotional well-being.

In contrast to change-based skills, acceptance-based skills in DBT encourage accepting reality and associated emotions without actively attempting to change them. A central acceptance-based skill in DBT is "radical acceptance." Radical acceptance promotes extreme experiential acceptance of reality in a nonevaluative way (Williams & Lynn, 2010). It is based on the notion that nonacceptance of painful reality that cannot be changed acts to increase suffering. Empirical evidence supports this notion by showing that nonacceptance emotion regulation strategies such as suppression and rumination are associated with elevated emotional distress (Aldao et al., 2010). As such, accepting reality may help by reducing suffering from what cannot be changed (Lynch et al., 2006; Williams & Lynn, 2010). Indeed, radical acceptance was rated as the most useful and meaningful distress tolerance skill practiced within DBT by patients and therapists (Jergensen, 2018; Kamody et al., 2020). Also, recent studies have shown that online training in radical acceptance can help healthy individuals cope with daily negative personal events and improve their emotion regulation abilities (Segal et al., 2023, 2025). In the context of war, where circumstances may be beyond one's control and outcomes unpredictable, cultivating radical acceptance may help in tolerating war-induced stress more effectively.

One population that could particularly benefit from targeted interventions for coping with war-related stress is women whose close family members have been deployed to war. Spousal deployment is associated with increased mental health challenges, including heightened depressive symptoms, anxiety, and acute stress reactions (Mansfield et al., 2010). Women with children face an even greater psychological burden, as they must manage both the emotional strain of their spouse's deployment and the demands of single parenthood (Braun-Lewensohn & Bar, 2017). Furthermore, in some conflicts, these women also experience the direct threat of war, further exacerbating their distress. Despite these significant challenges, this population remains largely understudied.

The current research compared the efficacy of change-based (TIP skills) versus acceptance-based (radical acceptance) DBT distress tolerance skills in reducing war-related emotional distress among women whose first-degree relatives or spouses were deployed due to war. The study was carried out shortly after the outbreak of the Israel–Hamas war on October 7, 2023. The study included an online intervention composed of a skill-learning workshop followed by three guided self-help home training sessions over 1 week.

Eligible participants were randomly assigned to a relaxation intervention condition involving learning and practicing TIP skills (TIP condition), a radical acceptance intervention condition (radical acceptance [RA] condition), or an active monitoring control condition. Measures of emotional distress including depression, stress, anxiety, and emotion regulation difficulties were collected at baseline, postintervention, and 3-week follow-up. Additionally, the home training sessions included monitoring daily stressors and implementing RA or TIP (the active control condition also monitored stressors during the home training sessions but did not implement any strategy). Data on negative and positive affect were assessed before and after each training session.

Our preregistered hypotheses were that (a) during the home training sessions, those in the RA and TIP conditions would show a significant pre-to-post session reduction in negative affect and an increase in positive affect compared with the control condition,

(b) those in the RA and TIP conditions would show lower levels of anxiety, depression, stress, and emotion regulation difficulties at postintervention compared with baseline. No such differences were expected among controls. Differences between RA and TIP were assessed on an exploratory basis, (c) at 3-week follow-up, the effects of the intervention would be maintained in the RA condition, but not in the TIP condition given that TIP skills focus more strongly on immediate relief of distress.

Method

Participants

We recruited 149 female adult participants (130 of whom were included in the final analysis) aged 21–66 ($M = 32.7$, $SD = 8.6$), who reported experiencing stress due to a first-degree relative or spouse drafted to the reserve army after the outbreak of the Israel–Hamas war on October 7, 2023 (see sample characteristics in Table 1). The recruitment began in November 2023 and all data were collected by February 2024. Inclusion criteria were (a) age over 18 years old, (b) having a first-degree family member or spouse absent from home due to army duty, (c) having access to a computer at home, and (d) currently living in Israel and fluent in Hebrew. Exclusion criteria included (a) a diagnosis of a psychiatric disorder (screened through self-report), (b) experiencing direct trauma as a result of the current war, (c) the loss of a family member or a close friend in the current war, and (d) current or past participation in DBT treatment.

The sample size was determined via a power analysis using G*Power (Faul et al., 2007), which showed that 40 participants per group would be sufficient to reveal within (Time)—between

(Condition) factor interactions for detecting pre-to-post intervention effects across the intervention conditions (i.e., Time $\times$ Condition interaction) in a repeated measures analysis of variance (ANOVA) with a power $>80\%$, a priori α set at .05, expecting a medium effect size of partial $\eta^2 = .06$. We decided to recruit 150 participants due to potential dropout and data loss.

Procedure

Participants were recruited via advertisements on social media (e.g., Facebook) and through online support groups created after the outbreak of the war. The advertisements reached out to women who “experience high levels of stress due to a spouse or first-degree family member deployment to war.” These women were invited to participate in an “online intervention for learning and practicing skills for coping with war-related stress.” All ads included a link to a form with general information about the study’s requirements and eligibility criteria, where interested individuals could provide their contact information. A research coordinator then contacted them via phone to provide further details and assess eligibility according to the inclusion and exclusion criteria (see consort diagram in Figure 1). Active enrollment occurred between November 23 and December 23, approximately 1–2 months after the outbreak of the war.

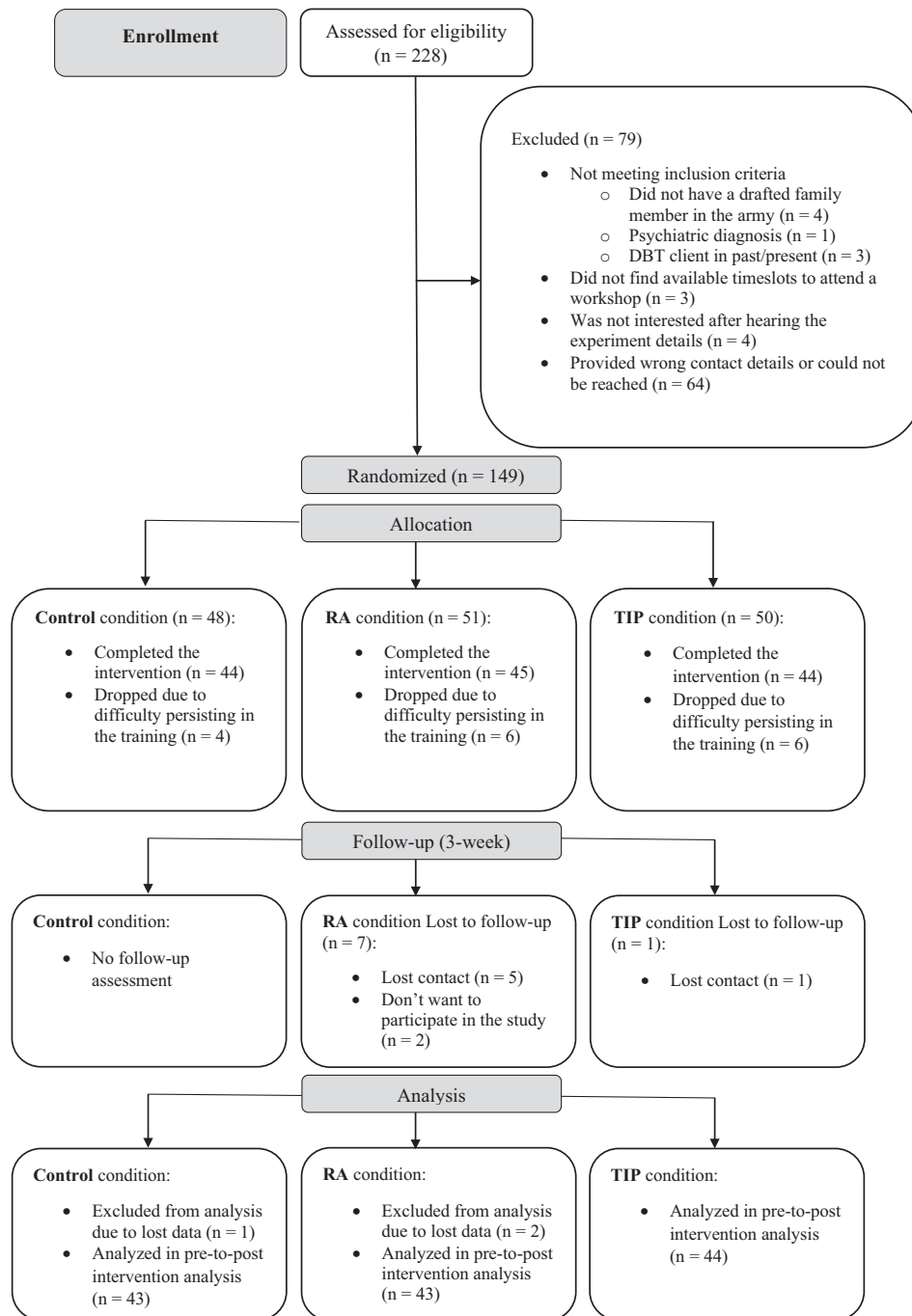
After signing informed consent, participants were randomized to one of the three experimental conditions (RA, TIP, or Control) using “Research Randomizer” at <https://www.randomizer.org/>. The general design of the study (depicted in Figure 2) involved baseline assessment of self-report questionnaires to assess levels of depression, stress, anxiety, and difficulties in emotion regulation (detailed below). Then, participants in the RA and TIP intervention conditions,

Table 1

Sample Characteristics (Counts [%] for Categorical and Mean [Standard Deviations] for Continuous Variables)

Baseline characteristic	Radical acceptance ($n = 43$)	TIP ($n = 44$)	Control ($n = 43$)	Full sample ($N = 130$)
Age	32.35 (8.36)	32.8 (8.94)	33 (7.25)	32.7 (8.6)
Deployed relative				
Son	1 (2.33%)	1 (2.27%)	0 (0%)	2 (1.54%)
Brother	1 (2.33%)	2 (4.54%)	2 (4.65%)	5 (3.85%)
Spouse	38 (88.37%)	33 (75%)	35 (81.39%)	106 (81.54%)
More than one relative	3 (6.98%)	8 (18.18%)	6 (13.95%)	17 (13.08%)
Highest educational level				
High school	8 (18.61%)	10 (22.73%)	5 (11.63%)	23 (17.69%)
Bachelor’s degree	24 (55.81%)	17 (38.64%)	27 (62.79%)	68 (52.31%)
Graduate’s degree	11 (25.58%)	17 (38.64%)	11 (25.58%)	39 (30%)
Income				
Well below average	10 (23.26%)	6 (13.64%)	11 (25.58%)	27 (20.77%)
Below average	15 (34.88%)	12 (27.27%)	10 (23.26%)	37 (28.46%)
Average	8 (18.6%)	14 (31.82%)	12 (27.91%)	34 (26.15%)
Above average	8 (18.6%)	12 (27.27%)	8 (18.6%)	28 (21.54%)
Well above average	2 (4.65%)	0 (0%)	2 (4.65%)	4 (3.08%)
Marital status				
Single	0 (0%)	0 (0%)	1 (2.32%)	1 (0.77%)
Married/partnered	43 (100%)	44 (100%)	42 (97.67%)	129 (99.23%)
Divorced/widowed	0 (0%)	0 (0%)	0 (0%)	0 (0%)
Other	0 (0%)	0 (0%)	0 (0%)	0 (0%)
Children				
Yes	26 (60.46%)	26 (59.09%)	26 (60.46%)	78 (60%)
No	17 (39.53%)	18 (40.91%)	17 (39.53%)	52 (40%)
Number of children	1.814 (1.82)	1.636 (1.92)	2.279 (2.58)	2.577 (1.13)

Note. All between conditions’ comparisons (one-way analysis of variance for continuous variables and χ^2 tests for categorical variables) are nonsignificant (all $ps > .30$). TIP = temperature, intense exercise, paced breathing or paired muscle relaxation.

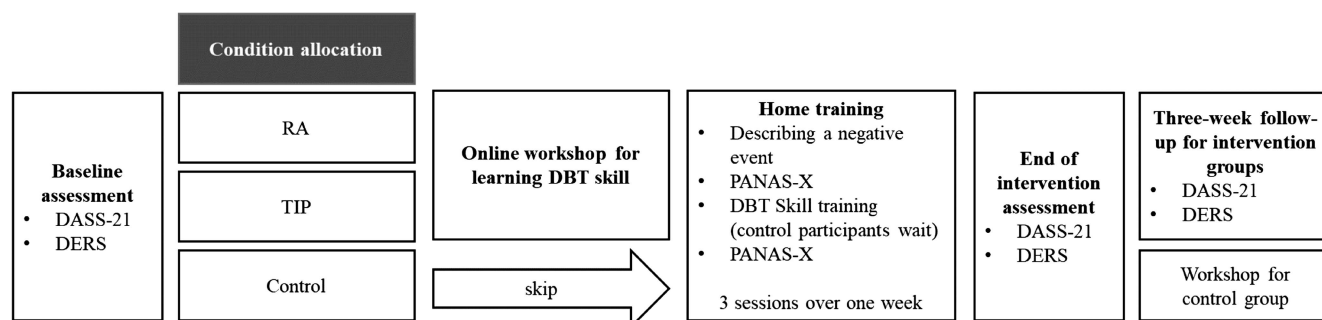
Figure 1*Flow Diagram of Enrollment, Allocation to Training Condition, Follow-Up, and Data Analysis*

Note. DBT = dialectical behavior therapy; RA = radical acceptance; TIP = temperature, intense exercise, paced breathing or paired muscle relaxation.

but not controls, participated in an online video-conference workshop (detailed below) to learn how to implement TIP or RA. Participants in all conditions, including controls, participated in three online home training sessions. At the end of the third home training session, an end-of-intervention (EOI) assessment was completed. After the EOI

assessment, those in the control condition were offered to participate in an RA or TIP workshop. Thus, a 3-week follow-up assessment was carried out only for those in the RA and TIP conditions. At the end of the study, participants were debriefed and received a monetary reward for the time taken to complete the questionnaires.

Figure 2
The Study Design



Note. DASS-21 = The Depression Anxiety Stress Scale 21; DERS = The Difficulties in Emotion Regulation Scale; RA = radical acceptance; TIP = temperature, intense exercise, paced breathing or paired muscle relaxation; PANAS-X = The Positive and Negative Affect Scale expanded form; DBT = dialectical behavior therapy

Online Workshops

Each workshop was led by two graduate students in the clinical psychology track. In line with the standard DBT skills training group format (Linehan, 2015), each workshop included a leading facilitator and a cofacilitator. Importantly, we counterbalanced the roles across the different workshops to ensure that the graduate students took turns serving as the leader and cofacilitator, preventing any single individual from leading all workshops. Facilitators' training included the completion of a semester-long DBT course and designated training for RA and TIP workshops delivered by a licensed psychiatrist (Helene Sher) and a clinical psychologist (Noam Weinbach), both experienced DBT therapists. The workshops were recorded for supervision purposes. The research team held weekly meetings for supervision and consultation, which included reviewing and discussing sections of the recorded workshops. Each workshop included 4–10 participants ($M = 7$, $SD = 1.8$) and lasted 75 min. The content of the RA and TIP workshops closely followed the DBT skills training manual (Linehan, 2015) with minor cultural and situational adjustments (e.g., working with examples related to the ongoing war).

Each workshop began with an introduction between participants and facilitators, followed by an explanation of the workshop guidelines and objectives. Then, facilitators provided theoretical background and psychoeducation regarding the skills being taught (RA or TIP). Following that, the facilitators discussed the practical implementation of the skill including joint practice and discussions. A summary of the content of each workshop and timing can be found in [Supplemental Methods A and B](#).

Home Training

All participants, including those in the control condition, were asked to complete three home training sessions over a period of 1 week (approximately one practice session every 2 days). These sessions were conducted online via Qualtrics in a guided self-help format without the involvement of the research staff. A link to each training session was sent to participants via email. At the beginning of each training session, participants were asked to write down a short paragraph describing a distressing event from the past week that still evokes negative emotions when recalled. After describing

the event, participants in the RA and TIP conditions implemented their acquired DBT skills to cope with the event using guided audio instructions (4:50 min). Specifically, participants in the RA condition listened to guided practice of “radical acceptance”, while participants in the TIP condition listened to a “paired muscle relaxation” practice from the DBT training manual. Participants in the control condition waited for 2 min after describing the event without practicing any skill. Negative and positive affect (measured by the The Positive and Negative Affect Scale expanded form) were assessed before and after skill implementation. Each training session lasted 10–15 min. If delayed, participants could reschedule within a 2-week window, but no longer. The average completion time was 6.58 days ($SD = 1.83$), with 78% completing training within 1 week, and no participant exceeding the 2-week limit.

Measures

Self-Report Questionnaires

Demographic Questionnaire. We collected data on participants' age, gender, income, marital status, number of children, and residential location.

Difficulties in Emotion Regulation Scale (Gratz & Roemer, 2004). The Difficulties in Emotion Regulation Scale (DERS) is a 36-item self-report questionnaire that assesses emotion regulation abilities. It provides a total score in addition to six subscales: (1) nonacceptance of emotional responses, (2) difficulties engaging in goal-directed behavior, (3) impulse control difficulties, (4) lack of emotional awareness, (5) limited access to emotion regulation strategies, (6) lack of emotional clarity. Cronbach's α in the present study was .92 at baseline, .93 at EOI, and 0.93 at follow-up. A Hebrew version of the DERS has shown good convergent and discriminant validity (Erez & Gordon, 2024).

The Depression Anxiety Stress Scale 21 (Lovibond & Lovibond, 1995). The Depression Anxiety Stress Scale 21 (DASS-21) is a 21-item self-report questionnaire that assesses anxiety, depression, and stress. It provides three subscales: (1) depression, (2) anxiety, and (3) stress. Cronbach's α in the present study was .86 for depression, .81 for anxiety, and .87 for stress at baseline. At EOI Cronbach's α was .84 for depression, .80 for

anxiety, and .86 for stress. At follow-up, Cronbach's α was .87 for depression, .83 for anxiety, and .91 for stress. The Hebrew version of the DASS-21 has shown good convergent and discriminant validity (Anghel, 2020).

The Positive and Negative Affect Scale Expanded Form (Clark & Watson, 1994). The Positive and Negative Affect Scale expanded form is a 60-item Self-Report Questionnaire that assesses current positive and negative emotional experiences. To reduce participant burden, we selected the 14 most relevant items, with seven items representing negative emotions (i.e., guilty, sad, irritable, afraid, lonely, alert, distressed) and seven items representing positive emotions (i.e., relaxed, energetic, confident, happy, calm, determined, enthusiastic). The scale has shown internal consistency ($\alpha = .95$) and test-retest reliability ($r = 0.78$; Stice et al., 2021). In the present study, average Cronbach's α across training sessions was .82 before skill implementation and 0.88 after skill implementation. The Hebrew version of the questionnaire has shown good convergent and discriminant validity (Ben-Zur, 2002; Segal et al., 2023).

Data Analysis

To estimate changes in participants' negative and positive affect during training sessions, we employed hierarchical linear modeling with restricted maximum likelihood estimation due to the nested structure of the data. Specifically, repeated measurements (i.e., before and after each of the three training sessions) formed our Level-1 units. These measurements were nested within individuals (Level-2 units). Independent variables were coded using effects-coding to facilitate interpretation of coefficients. Random intercepts and slopes were entered into the model for Level-1 independent variables, and "variance components" were used as the Level-2 covariance structure. Specifically, to examine changes in negative affect we ran a model with measurement (dichotomous Level-1 variable: pre vs. post), RA condition (dichotomous Level-2 variable: RA vs. control), TIP condition (dichotomous Level-2 variable: TIP vs. control), and both cross-level interactions (RA Condition $\times$ Measurement and TIP Condition $\times$ Measurement) as independent variables. Negative affect served as the dependent variable. To examine changes in positive affect we ran a model with the same independent variables but with positive affect as the dependent variable.

To assess pre-to-post intervention differences in self-report measures of emotional distress as a function of condition, we carried out a repeated measures ANOVA, with time (baseline/EOI) as a within-subject factor and condition (RA/TIP/Control) as a between-subject factor. The DERS total score and DASS-21 three subscales (depression, anxiety, and stress) were used as dependent variables in the different ANOVA tests. To assess the effects at 3-week follow-up, we carried out an additional repeated measures ANOVAs with time (EOI/follow-up) as a within-subject factor and condition (RA/TIP) as a between-subject factor. The DERS total score and DASS-21 three subscales (depression, anxiety, and stress) were used as dependent variables in the different ANOVA tests. These analyses were conducted using a per-protocol approach, which focuses on participants who completed the intervention, as we were primarily interested in assessing effects among those who fully completed the intervention. However, we also conducted an intent-to-treat analysis to account for potential biases due to dropouts (see Supplemental Results A). Importantly, all significant effects observed in the per-protocol analysis remained significant in the

intent-to-treat analysis. Partial η^2 was used as an effect size estimate with 0.01, 0.06, and 0.14, representing small, medium, and large effect sizes, respectively (Cohen, 1988).

Transparency and Openness

This study received ethics approval by the institutional review board at the University of Haifa (384/23). The study, its hypotheses and analysis plan, were preregistered at https://osf.io/tycgw/?view_only=a0c9618415d448b3bc4e54c8441aa3d5. We report how we determined our sample size, all data exclusions, all manipulations, and all measures in the study. The data used in the analyses and the code for conducting the analyses are available online at https://osf.io/tycgw/?view_only=a0c9618415d448b3bc4e54c8441aa3d5. There are no previous publications that resulted from the reported data set. While this study focused on emotional aspects of war-related distress, additional measures that are beyond the scope of the present study were collected and will be reported in future publications.

Results

Retention Rate and Missing Data

Retention rate was high and did not significantly differ between conditions ($\chi^2_{(2)} = 0.26, p = .88$), with 86.27% of the RA condition, 88% of the TIP condition, and 89.58% of the control condition completing the intervention. Additionally, 72.54% of the RA condition and 86% of the TIP condition completed the follow-up assessment with no significant difference ($\chi^2_{(1)} = 2.77, p = .10$). Four participants missed one home training session, accounting for 1% of the overall training sessions' data. For these four participants, we based our analyses on the two completed session scores.

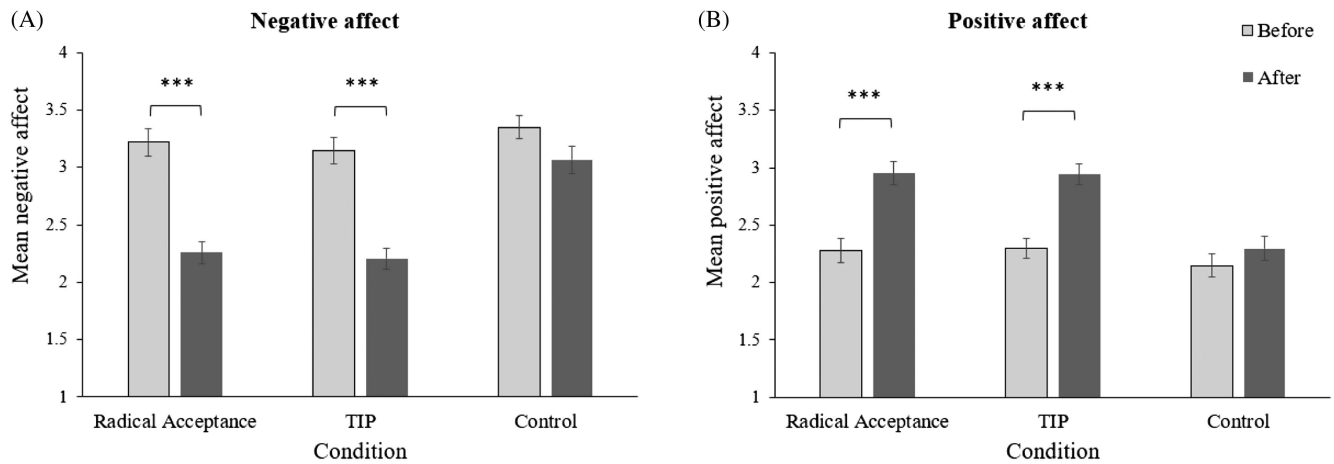
Changes in Negative and Positive Affect During Training Sessions

The analysis assessing pre-to-postsession changes in negative affect (see Figure 3A) revealed a significant RA Condition $\times$ Measurement interaction effect ($B = -0.23, SE = 0.06, t = -3.66, p < .001, 95\% \text{ CI } [-0.36, -0.11]$). This effect was such that the decrease in negative affect from pre- to postsession was greater in the RA condition (estimated $\Delta = 0.96$) compared with the control condition (estimated $\Delta = 0.28; B = -0.68, SE = 0.11, t = -6.23, p < .001, 95\% \text{ CI } [-0.90, -0.46]$). We also found a significant TIP Condition $\times$ Measurement interaction effect ($B = -0.22, SE = 0.06, t = -3.40, p = .001, 95\% \text{ CI } [-0.34, -0.09]$). The interaction was such that decreases in negative affect from pre- to postsession were greater in the TIP condition (estimated $\Delta = 0.94$) compared with the control condition (estimated $\Delta = 0.28; B = -0.66, SE = 0.11, t = -6.11, p < .001, 95\% \text{ CI } [-0.88, -0.45]$). Finally, there was no significant difference in the change in negative affect between the RA and TIP conditions ($B = 0.02, SE = 0.11, t = 0.16, p = .874, 95\% \text{ CI } [-0.20, 0.24]$).

The analysis assessing pre-to-post session changes in positive affect (see Figure 3B) revealed a significant RA Condition $\times$ Measurement interaction effect ($B = 0.18, SE = 0.05, t = 3.69, p < .001, 95\% \text{ CI } [0.09, 0.28]$). This effect was such that increases in positive affect from pre to post session were greater in the RA condition ($\Delta = 0.67$) compared with the control condition ($\Delta = 0.15; B = 0.52, SE = 0.09, t = 6.12, p < .001, 95\% \text{ CI } [0.35, 0.69]$). We also found a significant TIP Condition $\times$ Measurement interaction effect ($B = 0.16, SE = 0.05,$

Figure 3

Differences Between Intervention Conditions in Negative and Positive Affect From Pre-to-Post Training Sessions



Note. The y axis represents mean negative affect in (A) and mean positive affect in (B), based on the PANAS-X. The x axis represents the different conditions (radical acceptance, TIP, and control). Light gray bars represent presessions measurement, dark gray bars reflect postsessions measurement. PANAS-X = The Positive and Negative Affect Scale expanded form; TIP = temperature, intense exercise, paced breathing or paired muscle relaxation.

*** $p < .001$

$t = 3.15, p = .002, 95\% \text{ CI } [0.06, 0.25]$). The interaction was such that the increase in positive affect from pre- to postsession was greater in the TIP condition (estimated $\Delta = 0.64$) compared with the control condition (estimated $\Delta = 0.15$; $B = 0.49, SE = 0.09, t = 5.83, p < .001, 95\% \text{ CI } [0.33, 0.66]$). Finally, there was no significant difference in the change in positive affect between the RA and TIP conditions ($B = -0.03, SE = 0.09, t = -0.32, p = .749, 95\% \text{ CI } [-0.20, 0.14]$). A full report of all main effects and interactions for both negative and positive affect can be found in [Supplemental Results B](#).

Changes in Emotional Distress Measures From Baseline to End of Intervention

Changes in Stress Levels (DASS-21 Stress Score)

There were no main effects for Time, $F(1, 127) = 3.55, p = .062, \eta_p^2 = .027$, nor Condition, $F(2, 127) = 0.12, p = .888, \eta_p^2 = .002$. Importantly, the ANOVA revealed a significant Time $\times$ Condition interaction, $F(2, 127) = 6.25, p = .003, \eta_p^2 = .09$. In line with our hypotheses, planned comparisons revealed a significant medium-to-large effect indicating a decrease in stress levels from pre-to-post intervention among the participants in the RA condition, $F(1, 127) = 14.71, p < .001, \eta_p^2 = .104$, but not among controls, $F(1, 127) = 0.44, p = .512, \eta_p^2 = .003$. In contrast to the hypothesis, no pre-to-post changes were found in the TIP condition, $F(1, 127) = 0.00, p = .971, \eta_p^2 = .000$ (see [Figure 4](#)). An exploratory analysis did not reveal a significant difference between RA and TIP at EOI $F(1, 127) = 1.43, p = .233, \eta_p^2 = .011$.

Changes in Depression Levels (DASS-21 Depression Score)

The ANOVA revealed a significant main effect for Time, $F(1, 127) = 5.77, p = .018, \eta_p^2 = .043$, but no effect for Condition, $F(2, 127) = 0.35, p = .709, \eta_p^2 = .005$. Importantly, the Time $\times$ Condition

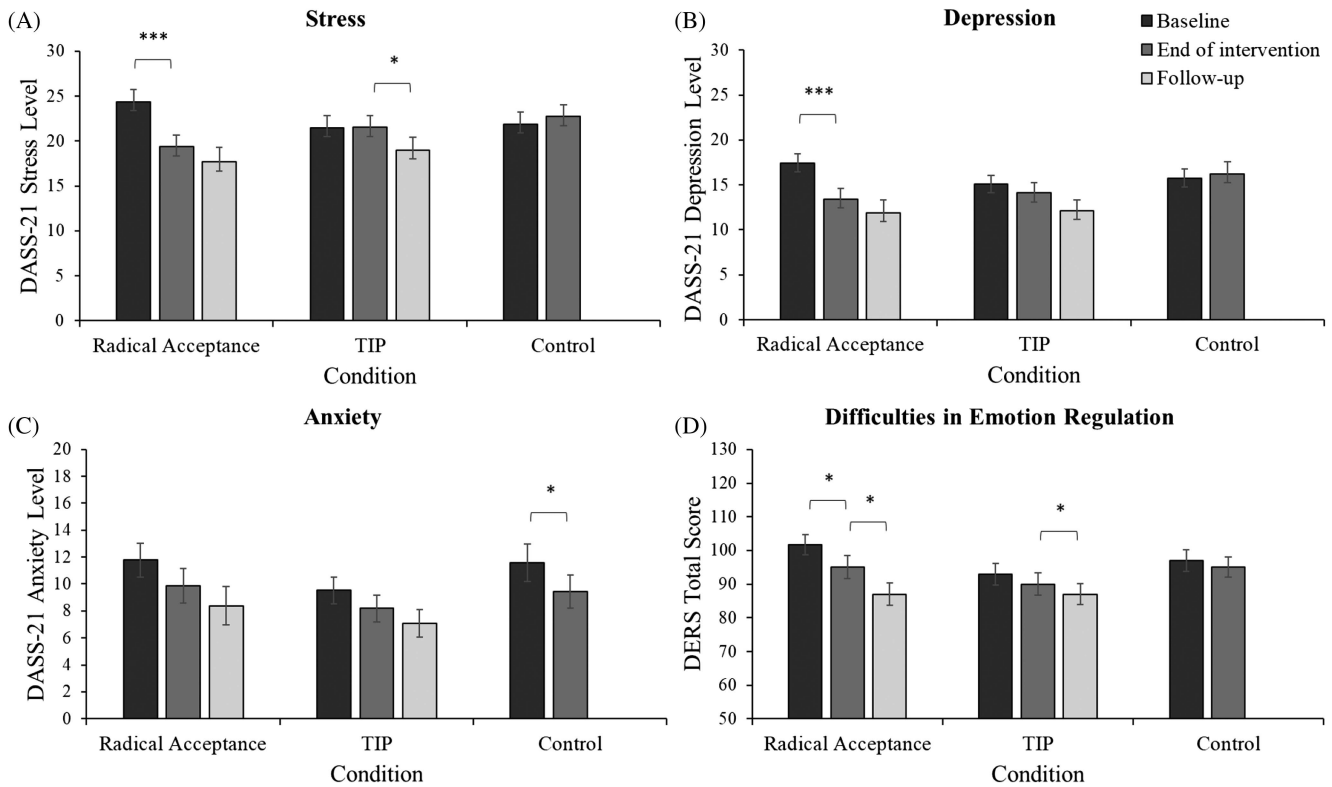
interaction was significant, $F(2, 127) = 4.33, p = .015, \eta_p^2 = .064$. In line with our hypotheses, planned comparisons revealed a significant medium-to-large effect demonstrating a decrease in depression levels from pre-to-post intervention, only among the participants in the RA condition, $F(1, 127) = 12.81, p < .001, \eta_p^2 = .091$, but not in the control condition, $F(1, 127) = 0.21, p = .646, \eta_p^2 = .001$, nor in the TIP condition, $F(1, 127) = 0.77, p = .385, \eta_p^2 = .006$. A between-groups contrast did not result in a difference at EOI between RA and TIP, $F(1, 127) = 0.13, p = .713, \eta_p^2 = .001$.

Changes in Anxiety Levels (DASS-21 Anxiety Score)

The ANOVA revealed a significant main effect for Time, $F(1, 127) = 9.98, p = .002, \eta_p^2 = .073$, but not Condition, $F(2, 127) = 0.97, p = .38, \eta_p^2 = .015$. In contrast to our expectation, the interaction between Condition and Time was not significant, $F(2, 127) = 0.19, p = .829, \eta_p^2 = .003$. Planned comparisons based on our preregistered hypotheses, showed a near significant small-to-medium effect indicating a pre-to-post intervention decrease in anxiety in the RA condition, $F(1, 127) = 3.54, p = .067, \eta_p^2 = .027$, a small-to-medium effect showing a significant decrease in the control condition, $F(1, 127) = 5.11, p = .029, \eta_p^2 = .039$, and a small and nonsignificant decrease in the TIP condition, $F(1, 127) = 1.81, p = .185, \eta_p^2 = .014$. No difference was found between RA and TIP at EOI, $F(1, 127) = 1.02, p = .313, \eta_p^2 = .008$.

Changes in Difficulties in Emotion Regulation (DERS Total Score)

The ANOVA revealed a significant main effect for Time, $F(1, 127) = 7.15, p = .008, \eta_p^2 = .053$, but not for Condition, $F(2, 127) = 1.34, p = .264, \eta_p^2 = .021$. In contrast to the hypothesis, the interaction between Condition $\times$ Time was not significant, $F(2, 127) = 1.12, p = .328, \eta_p^2 = .017$. However, planned comparisons based on our preregistered hypothesis showed a small-to-medium

Figure 4*Differences Between the Intervention Conditions in Emotional Distress From Baseline to EOI*

Note. The y axis represents stress levels based on the DASS-21 in (A), depression levels based on DASS-21 in (B), anxiety levels based on DASS-21 in (C), and difficulties in emotion regulation based on the Difficulties in Emotion Regulation Scale (DERS) in (D). The x axis represents the different conditions (radical acceptance, TIP, and control). Black bars represent baseline assessment, dark gray bars reflect EOI assessment, and light gray bars represent follow-ups which were assessed only in the radical acceptance and TIP conditions. EOI = end of intervention; DASS-21 = Depression Anxiety Stress Scale 21; TIP = temperature, intense exercise, paced breathing or paired muscle relaxation.

* $p < .05$. *** $p < .001$.

effect of a pre-to-post significant decrease in emotion regulation difficulties in the RA condition, $F(1, 127) = 5.76, p = .021, \eta_p^2 = .043$, but not in the TIP condition, $F(1, 127) = 1.33, p = .255, \eta_p^2 = .01$, nor in the control condition, $F(1, 127) = 0.83, p = .368, \eta_p^2 = .006$. No difference was found between RA and TIP at EOI, $F(1, 127) = 1.06, p = .302, \eta_p^2 = .008$.

Changes in Emotional Distress From EOI to Follow-Up (RA and TIP Conditions Only)

Analysis of EOI to follow-up differences in the main outcomes showed that in the RA condition, there were no differences between EOI and follow-up in levels of depression, $F(1, 77) = 1.19, p = .283, \eta_p^2 = .015$, anxiety, $F(1, 77) = 0.61, p = .441, \eta_p^2 = .008$, and stress, $F(1, 77) = 1.04, p = .315, \eta_p^2 = .013$. In contrast, there was a large effect showing a decrease in emotion regulation difficulties from EOI to follow-up, $F(1, 77) = 9.96, p = .003, \eta_p^2 = .115$. In the TIP condition, there were no EOI to follow-up differences in levels of depression, $F(1, 77) = 2.81, p = .101, \eta_p^2 = .035$, anxiety, $F(1, 77) = 1.46, p = .233, \eta_p^2 = .019$. Levels of stress and difficulties in emotion regulation decreased, as indicated by a small-to-medium and

medium effects, $F(1, 77) = 5.61, p = .023, \eta_p^2 = .068$ and $F(1, 77) = 6.78, p = .013, \eta_p^2 = .081$, respectively.

Discussion

The study examined the influence of a 1-week online intervention in DBT distress tolerance skills on war-related emotional distress among women. Specifically, we compared the effectiveness of an acceptance-based DBT skill (i.e., radical acceptance) versus a change-based DBT skill (i.e., TIP skills) versus an active control condition for alleviating short- and long-term emotional distress during war. Retention rates were high in both intervention conditions. During the home training sessions, participants in both the radical acceptance and TIP conditions successfully used these strategies for reducing negative affect and increasing positive affect after describing an emotionally distressing event. However, only those who trained in radical acceptance demonstrated a significant pre-to-post intervention decrease in stress (medium-to-large effect), depression (medium-to-large effect), and fewer difficulties in emotion regulation (small-to-medium effect). These effects remained at the 3-week follow-up.

Given the detrimental effects of living in war-afflicted regions on emotional well-being (Amone-P'olak et al., 2007; Charlson et al., 2019; Kurapov et al., 2023; Lapid Pickman et al., 2021; Miller & Rasmussen, 2010), there is a pressing need to identify brief, easily disseminated interventions to help individuals who cope with war-related stress. It has been suggested that providing emotion regulation tools can help individuals reduce emotional distress and foster adaptive coping during war (Gross et al., 2013; Punamäki et al., 2014). This study was the first to provide evidence that training in distress tolerance skills from a well-established treatment for emotion dysregulation, namely DBT, can effectively reduce emotional distress during active war among a war-affected population.

Findings from this study indicate that DBT skills promoting emotional acceptance (i.e., radical acceptance) and those focusing on body relaxation techniques (i.e., TIP skills) result in a similar reduction in negative affect and an increase in positive affect immediately after implementing them for coping with a personally distressing event. These effects were observed during home training sessions and were absent in the active control condition, which also described distressing events but did not apply any coping skills. Therefore, the observed benefits in the experimental conditions cannot be attributed to the act of sharing or documenting distressing thoughts and emotions, as this was also done in the control condition. More broadly, DBT distress tolerance skills were designed to address acute stress (Linehan, 2015), yet they are rarely studied as standalone strategies outside the context of comprehensive DBT. This study provides empirical evidence regarding their effectiveness and aligns with evidence suggesting that DBT skills can also benefit nonclinical populations (Mazza et al., 2016; Moore et al., 2018).

In examining pre-to-post intervention changes in the primary outcomes, we found that participants in the radical acceptance condition exhibited a reduction in levels of stress, depression, and difficulties in emotion regulation, but not anxiety. These effects were absent in the active control condition. Contrary to our expectations, participants in the TIP condition also did not show any pre-to-post intervention improvements. This suggests that while both radical acceptance and TIP skills provide immediate relief from distress, only radical acceptance fosters a more profound and lasting shift in how individuals approach emotional challenges. The lack of sustained effects in the TIP condition indicates that its benefits are primarily confined to short-term stress management through physiological regulation, without necessarily addressing thought patterns or perceptions that contribute to longer term emotional well-being. Overall, these findings align with prior research demonstrating the effectiveness of radical acceptance in reducing emotional suffering and enhancing psychological well-being (Kamody et al., 2020; Lynch et al., 2006; Segal et al., 2023; Williams & Lynn, 2010).

With regard to longer term effects of the intervention through a 3-week follow-up, the results showed that the benefits of radical acceptance were maintained through follow-up and difficulties in emotion regulation showed further improvement. However, since participants in the TIP condition also showed a decrease in emotion regulation difficulties and stress from postintervention to follow-up—and we did not collect follow-up data for the active control condition—it is difficult to rule out the effects of time. Therefore, while the data provide preliminary evidence of persisting effects, these findings should be validated in future research.

Previous research has established that among war-affected populations, women have increased vulnerability to the psychological

consequences of war (Kurapov et al., 2023; Murthy & Lakshminarayana, 2006) and that spousal deployment is an additional stressor that is associated with increased stress, depression, and anxiety (Mansfield et al., 2010). This study shows that radical acceptance training can be a clinically meaningful intervention to assist these women in managing war-related emotional distress. Indeed, in the present study, participants in all experimental conditions were characterized by moderate levels of depression and stress based on norms (Lovibond & Lovibond, 1995). However, at the end of the intervention, only those in the radical acceptance condition transitioned to mild levels of depression and moved from the upper threshold of moderate stress to the lower threshold. The other conditions remained at moderate levels at the end of the intervention. This demonstrates that radical acceptance training can be used as a clinically meaningful intervention in this population.

The study has clinical implications for promoting accessible mental health during crises. Given the widespread impact of war, which affects large portions of populations in war-affected areas on all sides of a conflict, brief, cost-effective interventions that can be disseminated rapidly and broadly are of great value. In that sense, online interventions hold great promise as they can be facilitated in areas that are hard to reach, as mobility may be limited during real-time crises. Previous studies have shown that cognitive behavior therapy and mindfulness-based digital mental health interventions hold promise in improving symptoms of distress among those who experienced war (Asanov et al., 2024; Hobfoll et al., 2016; Knaevelsrud et al., 2015; Rice et al., 2019; Wagner et al., 2012). However, most of these studies were conducted when the war was not ongoing and focused on individuals diagnosed with posttraumatic stress disorder. The present study adds above and beyond previous knowledge by showing that an online radical acceptance training intervention is an effective method for alleviating immediate emotional distress, reducing depression and stress levels, and improving emotion regulation difficulties among women who are currently under threat as a result of war and whose spouses (or close relatives) were deployed.

Several limitations should be acknowledged. First, as noted earlier, the study's ability to determine long-term effects is limited by the relatively short follow-up period of 3 weeks postintervention and the absence of follow-up data for the active control condition. A follow-up assessment for the control condition was not included because it would have required delaying their access to the intervention for 4 weeks. Given the participants' active efforts to seek help amid an ongoing war, we considered it ethically inappropriate to postpone their participation in a workshop beyond the 1-week monitoring period. However, this limited our ability to separate the effects of the interventions from the effects of time at follow-up. Additionally, the sample was restricted to women affected by deployment of a spouse or first-degree relative and most were from a similar ethnic background and had no recent exposure to trauma or loss due to the war, which may limit the generalizability of the findings to other populations. Future research should aim to include more diverse individuals and settings to better understand the broader applicability of the intervention.

In summary, our findings demonstrate the efficacy of both radical acceptance and TIP skills in reducing emotional distress immediately after their implementation, with only radical acceptance offering more substantial longer term benefits in reducing stress and

depression and improving emotion regulation. Given the prevalence of ongoing conflicts and crises worldwide, our data can provide a basis for developing practical interventions to support populations in conflict-affected areas. To further establish radical acceptance as an evidence-based intervention for alleviating war-related stress, future research should explore its effects on additional populations and contexts. Additionally, it is important to assess the potential of the intervention to prevent the future development of psychopathology that often emerges following war among highly affected individuals.

References

- Aldao, A., Nolen-Hoeksema, S., & Schweizer, S. (2010). Emotion-regulation strategies across psychopathology: A meta-analytic review. *Clinical Psychology Review*, 30(2), 217–237. <https://doi.org/10.1016/j.cpr.2009.11.004>
- Amone-P'olak, K., Garnefski, N., & Kraaij, V. (2007). Adolescents caught between fires: Cognitive emotion regulation in response to war experiences in Northern Uganda. *Journal of Adolescence*, 30(4), 655–669. <https://doi.org/10.1016/j.adolescence.2006.05.004>
- Anghel, E. (2020). Longitudinal invariance and information of the Depression, Anxiety, and Stress Scales. *Journal of Clinical Psychology*, 76(10), 1923–1937. <https://doi.org/10.1002/jclp.12968>
- Asanov, A.-M., Asanov, I., & Buenstorf, G. (2024). A low-cost digital first aid tool to reduce psychological distress in refugees: A multi-country randomized controlled trial of self-help online in the first months after the invasion of Ukraine. *Social Science & Medicine*, 362, Article 117442. <https://doi.org/10.1016/j.socscimed.2024.117442>
- Bankoff, S. M., Karpel, M. G., Forbes, H. E., & Pantalone, D. W. (2012). A systematic review of dialectical behavior therapy for the treatment of eating disorders. *Eating Disorders: The Journal of Treatment & Prevention*, 20(3), 196–215. <https://doi.org/10.1080/10640266.2012.668478>
- Ben-Zur, H. (2002). Coping, affect and aging: The roles of mastery and self-esteem. *Personality and Individual Differences*, 32(2), 357–372. [https://doi.org/10.1016/S0191-8869\(01\)00031-9](https://doi.org/10.1016/S0191-8869(01)00031-9)
- Braun-Lewensohn, O., & Bar, R. (2017). Coping and quality of life of soldiers' wives following military operation. *Psychiatry Research*, 254, 90–95. <https://doi.org/10.1016/j.psychres.2017.04.037>
- Charlson, F., van Ommeren, M., Flaxman, A., Cornett, J., Whiteford, H., & Saxena, S. (2019). New WHO prevalence estimates of mental disorders in conflict settings: A systematic review and meta-analysis. *The Lancet*, 394(10194), 240–248. [https://doi.org/10.1016/S0140-6736\(19\)30934-1](https://doi.org/10.1016/S0140-6736(19)30934-1)
- Clark, L. A., & Watson, D. (1994). *The PANAS-X: Manual for the positive and negative affect schedule—Expanded form*. <https://doi.org/10.17077/48vt-m4t2>
- Cohen, J. (1988). *Statistical power analysis for the behavioural sciences* (2nd ed.). Academic Press. <https://doi.org/10.4324/9780203771587>
- El Khoury-Malhame, M., Bou Malhab, S., Chaaya, R., Sfeir, M., & El Khoury, S. (2024). Coping during socio-political uncertainty. *Frontiers in Psychiatry*, 14, Article 1267603. <https://doi.org/10.3389/fpsy.2023.1267603>
- Erez, C., & Gordon, I. (2024). The imperfect yet valuable difficulties in Emotion Regulation Scale: Factor structure, dimensionality, and possible cutoff score. *Assessment*. Advance online publication. <https://doi.org/10.1177/10731911241261168>
- Faul, F., Erdfelder, E., Lang, A.-G., & Buchner, A. (2007). G*Power 3: A flexible statistical power analysis program for the social, behavioral, and biomedical sciences. *Behavior Research Methods*, 39(2), 175–191. <https://doi.org/10.3758/BF03193146>
- Goodman, M., Banthin, D., Blair, N. J., Mascitelli, K. A., Wilsnack, J., Chen, J., Messenger, J. W., Perez-Rodriguez, M. M., Triebwasser, J., Koenigsberg, H. W., Goetz, R. R., Hazlett, E. A., & New, A. S. (2016). A randomized trial of dialectical behavior therapy in high-risk suicidal veterans. *The Journal of Clinical Psychiatry*, 77(12), e1591–e1600. <https://doi.org/10.4088/JCP.15m10235>
- Gratz, K. L., & Roemer, L. (2004). Multidimensional assessment of emotion regulation and dysregulation: Development, factor structure, and initial validation of the difficulties in Emotion Regulation Scale. *Journal of Psychopathology and Behavioral Assessment*, 26(1), 41–54. <https://doi.org/10.1023/B:JOBA.0000007455.08539.94>
- Gross, J. J. (1999). Emotion regulation: Past, present, future. *Cognition and Emotion*, 13(5), 551–573. <https://doi.org/10.1080/026999399379186>
- Gross, J. J., Halperin, E., & Porat, R. (2013). Emotion regulation in intractable conflicts. *Current Directions in Psychological Science*, 22(6), 423–429. <https://doi.org/10.1177/0963721413495871>
- Harley, R., Sprich, S., Safren, S., Jacobo, M., & Fava, M. (2008). Adaptation of dialectical behavior therapy skills training group for treatment-resistant depression. *Journal of Nervous and Mental Disease*, 196(2), 136–143. <https://doi.org/10.1097/NMD.0b013e318162aa3f>
- Hobfoll, S. E., Blais, R. K., Stevens, N. R., Walt, L., & Gengler, R. (2016). Vets prevail online intervention reduces PTSD and depression in veterans with mild-to-moderate symptoms. *Journal of Consulting and Clinical Psychology*, 84(1), 31–42. <https://doi.org/10.1037/ccp0000041>
- Jergensen, K. (2018). Practice what you preach: An exploration of DBT therapists personal skill utilization in burnout prevention. *Clinical Social Work Journal*, 46(3), 187–199. <https://doi.org/10.1007/s10615-017-0633-6>
- Kamody, R. C., Thurston, I. B., & Burton, E. T. (2020). Acceptance-based skill acquisition and cognitive reappraisal in a culturally responsive treatment for binge eating in adolescence. *Eating Disorders: The Journal of Treatment & Prevention*, 28(2), 184–201. <https://doi.org/10.1080/10640266.2020.1731055>
- Knaevelsrud, C., Brand, J., Lange, A., Ruwaard, J., & Wagner, B. (2015). Web-based psychotherapy for posttraumatic stress disorder in war-traumatized Arab patients: Randomized controlled trial. *Journal of Medical Internet Research*, 17(3), Article e71. <https://doi.org/10.2196/jmir.3582>
- Koons, C. R., Robins, C. J., Lindsey Tweed, J., Lynch, T. R., Gonzalez, A. M., Morse, J. Q., Bishop, G. K., Butterfield, M. I., & Bastian, L. A. (2001). Efficacy of dialectical behavior therapy in women veterans with borderline personality disorder. *Behavior Therapy*, 32(2), 371–390. [https://doi.org/10.1016/S0005-7894\(01\)80009-5](https://doi.org/10.1016/S0005-7894(01)80009-5)
- Kurapov, A., Pavlenko, V., Drozdov, A., Bezliudna, V., Reznik, A., & Isralowitz, R. (2023). Toward an Understanding of the Russian-Ukrainian War Impact on University Students and Personnel. *Journal of Loss and Trauma*, 28(2), 167–174. <https://doi.org/10.1080/15325024.2022.2084838>
- Landes, S. J., Decker, S. E., McBain, S. A., Goodman, M., Smith, B. N., Sullivan, S. R., Spears, A. P., & Meyers, L. L. (2020). Dialectical behavior therapy research and program evaluation in the Department of Veterans Affairs. In J. Bedics (Ed.), *The handbook of dialectical behavior therapy: Theory, research, and evaluation* (pp. 275–292). Elsevier Academic Press. <https://doi.org/10.1016/B978-0-12-816384-9.00012-9>
- Lapid Pickman, L., Gelkopf, M., & Greene, T. (2021). Do positive and negative emotional reactions during war predict subsequent symptomatology? A prospective experience sampling study. *Journal of Anxiety Disorders*, 84, Article 102492. <https://doi.org/10.1016/j.janxdis.2021.102492>
- Linehan, M. M. (1993). *Cognitive-behavioral treatment of borderline personality disorder*. Guilford Press.
- Linehan, M. M. (2015). *DBT skills training manual* (2nd ed.). The Guilford Press.
- Linehan, M. M., & Wilks, C. R. (2015). The course and evolution of dialectical behavior therapy. *American Journal of Psychotherapy*, 69(2), 97–110. <https://doi.org/10.1176/appi.psychotherapy.2015.69.2.97>
- Lovibond, P. F., & Lovibond, S. H. (1995). The structure of negative emotional states: Comparison of the Depression Anxiety Stress Scales (DASS) with the Beck Depression and Anxiety Inventories. *Behaviour Research and Therapy*, 33(3), 335–343. [https://doi.org/10.1016/0005-7967\(94\)00075-U](https://doi.org/10.1016/0005-7967(94)00075-U)

- Lynch, T. R., Chapman, A. L., Rosenthal, M. Z., Kuo, J. R., & Linehan, M. M. (2006). Mechanisms of change in dialectical behavior therapy: Theoretical and empirical observations. *Journal of Clinical Psychology, 62*(4), 459–480. <https://doi.org/10.1002/jclp.20243>
- Malivoire, B. L. (2020). Exploring DBT skills training as a treatment avenue for generalized anxiety disorder. *Clinical Psychology: Science and Practice, 27*(4), Article e12339. <https://doi.org/10.1111/cpsp.12339>
- Mansfield, A. J., Kaufman, J. S., Marshall, S. W., Gaynes, B. N., Morrissey, J. P., & Engel, C. C. (2010). Deployment and the use of mental health services among U.S. Army wives. *The New England Journal of Medicine, 362*(2), 101–109. <https://doi.org/10.1056/NEJMoa0900177>
- Mazza, J. J., Dexter-Mazza, E. T., Miller, A. L., Rathus, J. H., & Murphy, H. E. (2016). *DBT? Skills in schools: Skills training for emotional problem solving for adolescents DBT steps-a*. Guilford Press.
- Meyers, L., Voller, E. K., McCallum, E. B., Thuras, P., Shallcross, S., Velasquez, T., & Meis, L. (2017). Treating veterans with PTSD and borderline personality symptoms in a 12-week intensive outpatient setting: Findings from a pilot program. *Journal of Traumatic Stress, 30*(2), 178–181. <https://doi.org/10.1002/jts.22174>
- Miller, K. E., & Rasmussen, A. (2010). War exposure, daily stressors, and mental health in conflict and post-conflict settings: Bridging the divide between trauma-focused and psychosocial frameworks. *Social Science & Medicine, 70*(1), 7–16. <https://doi.org/10.1016/j.socscimed.2009.09.029>
- Moore, K. E., Folk, J. B., Boren, E. A., Tangney, J. P., Fischer, S., & Schrader, S. W. (2018). Pilot study of a brief dialectical behavior therapy skills group for jail inmates. *Psychological Services, 15*(1), 98–108. <https://doi.org/10.1037/ser0000105>
- Murthy, R. S., & Lakshminarayana, R. (2006). Mental health consequences of war: A brief review of research findings. *World Psychiatry, 5*(1), 25–30.
- Panos, P. T., Jackson, J. W., Hasan, O., & Panos, A. (2014). Meta-analysis and systematic review assessing the efficacy of Dialectical Behavior Therapy (DBT). *Research on Social Work Practice, 24*(2), 213–223. <https://doi.org/10.1177/1049731513503047>
- Priebe, S., Jankovic Gavrilovic, J., Bremner, S., Ajdukovic, D., Franciskovic, T., Galeazzi, G. M., Kucukalic, A., Lecic-Tosevski, D., Morina, N., Popovski, M., Schützwohl, M., & Bogic, M. (2013). Psychological symptoms as long-term consequences of war experiences. *Psychopathology, 46*(1), 45–54. <https://doi.org/10.1159/000338640>
- Punamäki, R.-L., Peltonen, K., Diab, M., & Qouta, S. R. (2014). Psychosocial interventions and emotion regulation among war-affected children: Randomized control trial effects. *Traumatology, 20*(4), 241–252. <https://doi.org/10.1037/h0099856>
- Rice, V. J., Liu, B., Allison, S. C., & Schroeder, P. J. (2019). Mindfulness training offered in-person and in a virtual world—Weekly self-reports of stress, energy, pain, and sleepiness among us military active duty and veteran personnel. *Mindfulness, 10*(9), 1815–1827. <https://doi.org/10.1007/s12671-019-01129-3>
- Segal, O., Bronshtein, K., & Weinbach, N. (2025). The benefits of radical acceptance of reality as a standalone strategy for emotion regulation. *Current Psychology, 44*(3), 1780–1792. <https://doi.org/10.1007/s12144-025-07286-0>
- Segal, O., Sher, H., Aderka, I. M., & Weinbach, N. (2023). Does acceptance lead to change? Training in radical acceptance improves implementation of cognitive reappraisal. *Behaviour Research and Therapy, 164*, Article 104303. <https://doi.org/10.1016/j.brat.2023.104303>
- Stice, E., Bohon, C., Gau, J. M., & Rohde, P. (2021). Factors that predict persistence versus non-persistence of eating disorder Symptoms: A prospective study of high-risk young women. *Behaviour Research and Therapy, 144*, 103932. <https://doi.org/10.1016/j.brat.2021.103932>
- Wagner, B., Schulz, W., & Knaevelsrud, C. (2012). Efficacy of an internet-based intervention for posttraumatic stress disorder in Iraq: A pilot study. *Psychiatry Research, 195*(1–2), 85–88. <https://doi.org/10.1016/j.psychres.2011.07.026>
- Williams, J. C., & Lynn, S. J. (2010). Acceptance: An historical and conceptual review. *Imagination, Cognition and Personality, 30*(1), 5–56. <https://doi.org/10.2190/IC.30.1.c>

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